جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : احمد محمد عبدالفتاح احمد
المحافظة : القاهره **الحي / المدينة :** حدائق القبه **العنوان :** القاهره
رقم البطاقة : 28303110100556 **رقم التليفون :** 01114587598 **تاريخ تسجيل الإستمارة :** 15-10-2026
العمر(تاريخ الميلاد) : 11-03-1983 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : اورام ليمفاويه
نتائج الأبحاث : (مرفق)
التشخيص النهائي : اورام ليمفاويه
خطة العلاج : أورام كيماوي العلاج بعقار (Rituximab 700mg) بواقع جرعه كل 3 اسابيع بالحقن الوريدي (متضمنا للعلاج الكيماوي) CHOP OR CVP [14284] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنه و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: احمد محمد عبدالفتاح احمد
National ID: 28303110100556

GUSTAVE ROUSSY INTERNATIONAL

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشان تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

A patient of lymphoma for Rituximab plus CHOP or CVP chemotherapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية

بطاقة تحقيق الشخصية

احمد

محمد عبدالفتاح احمد

١٥ ش ترعة القبه-مدينة ناصر

حدائق القبة - القاهره

٢٨٣٠٣١١٠١٠٠٥٦

LU6220618



٢٨٣٠٣١١٠١٠٠٥٥٦ ٢٠٢٦/٠٦

مهندس كهرباء

ذكر مسلم

١ البطاقة سارية حتى ٢٠٣٣/٠٦/١٣



Pat. Name: Ahmed Mohamed Abdel Fattah	Pat. ID: 13712122
Procedure Name: Pelviabdominal US	Date: 16/09/2026
Ref. Doctor: Bedon Bedon	

PELVI-ABDOMINAL ULTRASOUND

* **Liver**: normal size, homogeneous echopattern. No focal lesion or dilated biliary radicles. Portal vein is attenuated with faint color flow seen within "sequlae of old recanalized portal thrombosis" showing periportal cavernous transformation. Small simple right lobe hepatic cyst measuring 1.5x1 cm
* **Gall bladder**: good distension and normal wall thickness. Gall stone seen measuring 1.5 cm CBD is not dilated.
* **Spleen**: enlarged in size 23 cm showing homogeneous echopattern. Splenic vein is not dilated.
* **Both kidneys**: Normal site, size, cortex parenchymal thickness and echogenicity. No stones, backpressure or focal lesion.
* **Pancreas**: no abnormality seen.
* **Urinary bladder**: good distension and normal wall thickness. No stones, masses or diverticula.
* **Prostate**: average in size, homogeneous echopattern and intact capsule. Normal seminal vesicles.
* **No ascites, collection or mass in the abdominal or pelvic cavity.**

➢ **Conclusion:-**

- *Attenuated portal vein as sequlae of old portal thrombosis with portal HTN changes*
- *Simple hepatic cyst*
- *Splenomegaly*
- *Increased gaseous distention*

Much obliged;
Dr. Eslam Hamdy, M.Sc.

Signature Dr.Islam.Mohamed.Hamdy.Abdo
16/09/2026 09:55 م

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دكتور
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ت: 26904454 / 26904459

COMPLETE BLOOD COUNTS

Name	Patient No	Ref. No	Date	Requested By:
MR. AHMAD MOHAMMAD ABDEL FATTAH	491886	491886	25/02/2026	PROF. SAMEH EMARA

HAEMOGLOBIN		LEUCOCYTES			RBC's MORPHOLOGY		COMMENT
14.5 g/dl		Total Count 4.1 x 10^3 / µl					Normal haemoglobin level.
RBC's Count	6.9 x 10^6 / µl	Staff Neutrophils	2	%	Hypochromia	+	Moderate erythrocytosis. and hypochromic red cells.
Hct	43.6 %	Segmented Neutrophils	57	%	Microcytes	+	
		Lymphocytes	28	%	Macrocytes	-	Mild lymphocytopenia (= 1148 / ul).
MCV	67.2 fl (NR:78-97)	Monocytes	9	%	Anisocytosis	++	
		Eosinophils	4	%	Poikilocytosis	+	
MCH	22.3 pg (NR:26.5-32)	Basophils	0	%	Polychromasia	-	Normal platelets.
MCHC	33.3 % (NR:31.5-36)				Reticulocytes: 1.4 %		Normal ESR.
					Corrected: %		
RDW	19.8 % (NR:11.5-14.5)				Normoblasts /100 WBC's		
					HAEMOPARASITES Absent		

PLATELETS Count : 349,000 /mm^3 (NR : 150, 000 - 400, 000)
In film : Normal morphology

ESR drop per hour 1 mm

Code : – Absent + Mild ++ Moderate +++ Marked

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EXAMINATION OF BONE MARROW ASPIRATION

The iliac marrow of this patient shows moderately hypercellular particles with less cellular trails (possible associated fibrosis) . Granulocytopoiesis is active and shows normal morphology and normal maturation sequence. Myeloblasts are 1% of the nucleated marrow elements.

Erythropoiesis is active and shows predominance of the more mature precursors with mild dysplastic changes.

The myeloid : erythroid ratio is decreased (1.7 : 1).

Lymphocytes are normal in count (18%). No lymphoblasts. No lymphoma cells.

Megakaryocytes are abundant with a majority of unusual large forms with very hyperlobulated "bulbous" nuclei. Free platelets are increased in count occurring in large sheets.

Plasma cells are normal.

No haemoparasites.

No leukaemic, lymphomatous or nonhaematopoietic neoplastic infiltrations.

In sight of the bone marrow findings, JAK2 positivity and the history given by the patient, the most fitting diagnosis will be polycythaemia vera with starting transition to myelofibrosis.

Trephine marrow biopsy is separately reported.

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Name	Patient No	Ref. No	Date	Requested By:
MR. AHMAD MOHAMMAD ABDEL FATTAH	491886	491886	25/02/2026	PROF. SAMEH EMARA

BONE MARROW CORE BIOPSY EXAMINATION

Specimen: 2.1 cm core biopsy from left posterior iliac bone (Block ID: 26-487).

Comment: The bone marrow is moderately hypercellular for age (average cellularity ~ 90%).
Granulocytopoiesis is active with orderly sequence of maturation.
The erythroid series is stimulated with normoblastic maturation.
Megakaryocytes are markedly increased and are variable in size with frequent large sized forms with
hyperlobulated nuclei " cloud-shaped". Clustering is evident.
No abnormal lymphoid or plasma cells clusters.
An element of fibrous tissue deposition is present.
No evidence of granuloma, or abnormal cellular infiltration.

Conclusion: This bone marrow biopsy was done to a patient with history suggestive of polycythaemia vera.
The present picture represents starting transition to myelofibrosis (Post PV myelofibrosis).
Silver staining is recommended.

One stained slide was given to the patient.

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Name	Patient No	Ref. No	Date	Requested By
MR. AHMAD MOHAMMAD ABDEL FATTAH	491886	491886	25/02/2026	PROF. SAMEH EMARA

BLOOD EXAMINATION

Prothrombin Time (PT):	**Patient:**	**20**	Seconds	
	Control:	**11.2**	Seconds	
	Concentration	**56**	%	
	INR	**1.9**		
Partial Thromboplastin time (aPTT):	**Patient:**	**66**	Seconds	(NR: 30-42 seconds)

BLOOD PATHOLOGY REPORT Page 1 of 1

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COMPLETE BLOOD COUNTS

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RBC's Count	6.9 x 10^6 / µl	Staff Neutrophils	2 %	Hypochromia	+
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Hct	43.6 %	Lymphocytes	28 %	Macrocytes	-
		Monocytes	9 %	Anisocytosis	++
MCV	67.2 fl (NR:78-97)	Eosinophils	4 %	Poikilocytosis	+
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MCH	22.3 pg (NR:26.5-32)				
MCHC	33.3 % (NR:31.5-36)			Reticulocytes: 1.4 %	
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				HAEMOPARASITES Absent	
PLATELETS Count : 349,000 /mm^3 (NR : 150, 000 - 400, 000) In film : Normal morphology				ESR drop per hour 1 mm	

COMMENT

Normal haemoglobin level.

Moderate erythrocytosis. and hypochromic red cells.

Mild lymphocytopenia (= 1148 / ul).

Normal platelets.

Normal ESR.

Code : – Absent + Mild ++ Moderate +++ Marked

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The myeloid : erythroid ratio is decreased (1.7 : 1).

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Trephine marrow biopsy is separately reported.

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BONE MARROW CORE BIOPSY EXAMINATION

Specimen: 2.1 cm core biopsy from left posterior iliac bone (Block ID: 26-487).

Comment: The bone marrow is moderately hypercellular for age (average cellularity ~ 90%).
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No abnormal lymphoid or plasma cells clusters.
An element of fibrous tissue deposition is present.
No evidence of granuloma, or abnormal cellular infiltration.

Conclusion: **This bone marrow biopsy was done to a patient with history suggestive of polycythaemia vera.**
The present picture represents starting transition to myelofibrosis (Post PV myelofibrosis).
Silver staining is recommended.

One stained slide was given to the patient.

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Name	Patient No	Ref. No	Date	Requested By
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BLOOD EXAMINATION

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الإسم: السيد / احمد محمد عبد الفتاح
رقم المريض:59192 رقم المعمل:59192
التاريخ: 23/02/2026
الطبيب المعالج: ا . د / سامح عماره

MOLECULAR GENETICS UNIT

Test: Molecular Detection of JAK2 V617F/G1849T mutation in genomic DNA

Method: Real Time PCR

Specimen: Peripheral blood

Result: **JAK2 V617F Heterozygous mutation has been detected**

Comment:
Janus kinase 2 (JAK2) is a member of the family of JAK protein kinases involved in cytokine receptor signaling. An acquired somatic mutation in exon 14 of JAK2 (G > T) leads to substitution of valine to phenylalanine at codon 617 (V617F) resulting in constitutive activation of JAK2 independent of cytokines. The mutation status of JAK2 gene at this codon is considered to have diagnostic value in chronic myeloproliferative neoplasms (MPN). Reports indicate that it is present in 65-97% of polycythemia vera (PV), 23-57% of essential thrombocythemia (ET), 35-75% of idiopathic myelofibrosis (MF) and rarely in other Philadelphia-negative MPN.
A positive result identifies the JAK2 V617F mutation and is strongly supportive of PV, ET or MF. However, a positive result is not specific for a particular MPN and clinical pathological data are necessary for additional classification. A negative result does not exclude the presence of MPN or other neoplastic process.

Signature
Dr. Ahmed Ihab
Professor of Human Genetics





جمهورية مصر العربية

بطاقة تحقيق الشخصية

احمد

محمد عبدالفتاح احمد

١٥ ش ترعة القبه-مدينة ناصر

حدائق القبة - القاهره

٢٨٣٠٣١١٠١٠٠٥٦

LU6220618

National ID - Front

[[DOCTYPE_ID_FRONT]]





٢٨٣٠٣١١٠١٠٠٥٥٦ ٢٠٢٦/٠٦

مهندس كهرباء

ذكر مسلم

١ البطاقة سارية حتى ٢٠٣٣/٠٦/١٣



National ID – Back
[[DOCTYPE_ID_BACK]]

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One stained slide was given to the patient.

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بكلية الطب و مستشفيات جامعة عين شمس
120 شارع الثورة - مصر الجديدة
ت: 26904454 / 26904459

Name	Patient No	Ref. No	Date	Requested By
MR. AHMAD MOHAMMAD ABDEL FATTAH	491886	491886	25/02/2026	PROF. SAMEH EMARA

BLOOD EXAMINATION

Prothrombin Time (PT):	**Patient:**	**20**	Seconds	
	Control:	**11.2**	Seconds	
	Concentration	**56**	%	
	INR	**1.9**		
Partial Thromboplastin time (aPTT):	**Patient:**	**66**	Seconds	(NR: 30-42 seconds)

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COMPLETE BLOOD COUNTS

Name	Patient No	Ref. No	Date	Requested By:
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HAEMOGLOBIN		LEUCOCYTES			RBC's MORPHOLOGY		COMMENT
14.5 g/dl		Total Count 4.1 x 10^3 / µl					Normal haemoglobin level.
RBC's Count	6.9 x 10^6 / µl	Staff Neutrophils	2	%	Hypochromia	+	Moderate erythrocytosis. and hypochromic red cells.
		Segmented Neutrophils	57	%	Microcytes	+	
Hct	43.6 %	Lymphocytes	28	%	Macrocytes	-	Mild lymphocytopenia (= 1148 / ul).
		Monocytes	9	%	Anisocytosis	++	
MCV	67.2 fl (NR:78-97)	Eosinophils	4	%	Poikilocytosis	+	Normal platelets.
		Basophils	0	%	Polychromasia	-	
MCH	22.3 pg (NR:26.5-32)						Normal ESR.
MCHC	33.3 % (NR:31.5-36)				Reticulocytes: 1.4 %		
					Corrected: %		
RDW	19.8 % (NR:11.5-14.5)				Normoblasts /100 WBC's		
					HAEMOPARASITES Absent		
PLATELETS Count : 349,000 /mm^3 (NR : 150, 000 - 400, 000) In film : Normal morphology					ESR drop per hour 1 mm		

Code : – Absent + Mild ++ Moderate +++ Marked

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EXAMINATION OF BONE MARROW ASPIRATION

The iliac marrow of this patient shows moderately hypercellular particles with less cellular trails (possible associated fibrosis) . Granulocytopoiesis is active and shows normal morphology and normal maturation sequence. Myeloblasts are 1% of the nucleated marrow elements.

Erythropoiesis is active and shows predominance of the more mature precursors with mild dysplastic changes.

The myeloid : erythroid ratio is decreased (1.7 : 1).

Lymphocytes are normal in count (18%). No lymphoblasts. No lymphoma cells.

Megakaryocytes are abundant with a majority of unusual large forms with very hyperlobulated "bulbous" nuclei. Free platelets are increased in count occurring in large sheets.

Plasma cells are normal.

No haemoparasites.

No leukaemic, lymphomatous or nonhaematopoietic neoplastic infiltrations.

In sight of the bone marrow findings, JAK2 positivity and the history given by the patient, the most fitting diagnosis will be polycythaemia vera with starting transition to myelofibrosis.

Trephine marrow biopsy is separately reported.

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BONE MARROW CORE BIOPSY EXAMINATION

Specimen: 2.1 cm core biopsy from left posterior iliac bone (Block ID: 26-487).

Comment: The bone marrow is moderately hypercellular for age (average cellularity ~ 90%).
Granulocytopoiesis is active with orderly sequence of maturation.
The erythroid series is stimulated with normoblastic maturation.
Megakaryocytes are markedly increased and are variable in size with frequent large sized forms with hyperlobulated nuclei " cloud-shaped". Clustering is evident.
No abnormal lymphoid or plasma cells clusters.
An element of fibrous tissue deposition is present.
No evidence of granuloma, or abnormal cellular infiltration.

Conclusion: This bone marrow biopsy was done to a patient with history suggestive of polycythaemia vera. The present picture represents starting transition to myelofibrosis (Post PV myelofibrosis). Silver staining is recommended.

One stained slide was given to the patient.

BLOCK TYPE BONE MARROW ADMINISTRATION
Bone Marrow Aspiration

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الإسم: السيد / احمد محمد عبد الفتاح
رقم المريض:59192 رقم المعمل:59192
التاريخ: 23/02/2026
الطبيب المعالج: ا . د / سامح عماره

MOLECULAR GENETICS UNIT

Test: Molecular Detection of JAK2 V617F/G1849T mutation in genomic DNA

Method: Real Time PCR

Specimen: Peripheral blood

Result: **JAK2 V617F Heterozygous mutation has been detected**

Comment:

Janus kinase 2 (JAK2) is a member of the family of JAK protein kinases involved in cytokine receptor signaling. An acquired somatic mutation in exon 14 of JAK2 (G > T) leads to substitution of valine to phenylalanine at codon 617 (V617F) resulting in constitutive activation of JAK2 independent of cytokines. The mutation status of JAK2 gene at this codon is considered to have diagnostic value in chronic myeloproliferative neoplasms (MPN). Reports indicate that it is present in 65-97% of polycythemia vera (PV), 23-57% of essential thrombocythemia (ET), 35-75% of idiopathic myelofibrosis (MF) and rarely in other Philadelphia-negative MPN.

A positive result identifies the JAK2 V617F mutation and is strongly supportive of PV, ET or MF. However, a positive result is not specific for a particular MPN and clinical pathological data are necessary for additional classification. A negative result does not exclude the presence of MPN or other neoplastic process.

Signature

Dr. Ahmed Ihab
Professor of Human Genetics



Pat. Name: Ahmed Mohamed Abdel Fattah	Pat. ID: 13712122
Procedure Name: Pelviabdominal US	Date: 16/09/2026
Ref. Doctor: Bedon Bedon	

PELVI-ABDOMINAL ULTRASOUND

* **Liver**: normal size, homogeneous echopattern. No focal lesion or dilated biliary radicles. Portal vein is attenuated with faint color flow seen within "sequlae of old recanalized portal thrombosis" showing periportal cavernous transformation. Small simple right lobe hepatic cyst measuring 1.5x1 cm
* **Gall bladder**: good distension and normal wall thickness. Gall stone seen measuring 1.5 cm CBD is not dilated.
* **Spleen**: enlarged in size 23 cm showing homogeneous echopattern. Splenic vein is not dilated.
* **Both kidneys**: Normal site, size, cortex parenchymal thickness and echogenicity. No stones, backpressure or focal lesion.
* **Pancreas**: no abnormality seen.
* **Urinary bladder**: good distension and normal wall thickness. No stones, masses or diverticula.
* **Prostate**: average in size, homogeneous echopattern and intact capsule. Normal seminal vesicles.
* **No ascites, collection or mass in the abdominal or pelvic cavity.**

➢ **Conclusion:-**

- *Attenuated portal vein as sequlae of old portal thrombosis with portal HTN changes*
- *Simple hepatic cyst*
- *Splenomegaly*
- *Increased gaseous distention*

Much obliged;
Dr. Eslam Hamdy, M.Sc.